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REVIEW

Logotherapy and the Question of Evidence

A Narrative Review of the State of Research, Methodological Challenges, and Perspectives

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ABSTRACT

Logotherapy is one of the most established psychotherapeutic approaches of the twentieth century, yet it is rarely recognized as a distinct treatment within contemporary evidence-based psychotherapy. This narrative review examines how the current state of empirical research on logotherapeutic and meaning-centered interventions can be characterized and which methodological factors account for its comparatively narrow evidence base. First, it is argued that logotherapy comprises both an anthropological framework and a set of specific techniques, which complicates its operationalization as a testable treatment package. Second, central lines of evidence are summarized: the extensive, largely correlational research using meaning-in-life questionnaires, the meta-analytic integration of existential therapies, and randomized controlled trials of manualized meaning-centered programs in psycho-oncology. Overall, the findings suggest favorable effects of structured meaning-centered interventions but remain limited with respect to target populations, comparison conditions, and generalizability. Finally, the tension between the paradigm of disorder-specific efficacy trials and the contextual model of psychotherapy is discussed, together with the potential of process-oriented research that asks about mechanisms of change rather than school affiliation. The paper argues for a research agenda that respects anthropological assumptions while remaining methodologically compatible with mainstream psychotherapy research.

Keywords: logotherapy · evidence-based psychotherapy · meaning-centered psychotherapy · psychotherapy research · narrative review

1 Introduction

Since the 1990s, an understanding of evidence has become established in clinical psychology that judges psychotherapeutic procedures primarily by whether their efficacy for circumscribed disorders has been demonstrated in controlled studies (Chambless & Hollon, 1998). In the German-speaking countries, the work of Grawe et al. (1994) decisively shaped this development by programmatically seeking to lead psychotherapy "from confession to profession." Approaches that define themselves primarily through an image of the human being rather than through a disorder-specific treatment protocol easily come under pressure to justify themselves within this framework.

Logotherapy, founded by Viktor E. Frankl, is an instructive example of this. It has been received in numerous languages, has had a lasting influence on humanistic and existential psychotherapy, and is present in counseling, pastoral care, and palliative care. At the same time, it is hardly listed as an independent procedure in guidelines and textbooks of evidence-based psychotherapy. The present narrative review does not seek to resolve this discrepancy prematurely but rather to reconstruct its reasons. It draws on a selective choice of literature guided by its relevance to the research question and makes no claim to systematic completeness.

2 Logotherapy Between Anthropology and Treatment Procedure

Frankl himself understood logotherapy not as a procedure competing with other psychotherapies but as a supplement to them (Frankl, 1946). At its center is an image of the human being that assumes, in addition to the somatic and psychological dimensions, a spiritual, "noetic" dimension. From this dimension, Frankl derives freedom, responsibility, and the "will to meaning" as the primary motivation (Frankl, 1969). Accordingly, psychological suffering can arise, among

other things, when this will to meaning is frustrated; in this context, Frankl speaks of "noogenic neuroses" and of a widespread "existential vacuum."

In addition to this anthropological foundation, Frankl described specific techniques, in particular paradoxical intention for the treatment of anticipatory anxiety and obsessive-compulsive symptoms, and dereflection for excessive self-observation (Frankl, 1960). Later representatives have developed the methodology further, for instance in the sense of a modification of attitudes or of Socratic dialogue, and existential analysis has differentiated into an independent, more phenomenologically oriented procedure (Längle, 2013).

For research, this gives rise to a fundamental problem: it is not readily determinable what exactly is to be tested. If "logotherapy" is understood as an attitude that permeates any given treatment, it can hardly be delineated from a control condition. If, by contrast, it is reduced to individual techniques, it loses precisely those properties that its representatives consider central. This tension runs through the entire debate on evidence.

3 The State of Empirical Research

A first line of research concerns not the efficacy of logotherapy but the validity of its basic assumptions. Crumbaugh and Maholick (1964) had already developed the Purpose in Life Test, an instrument designed to make Frankl's concept of the existential vacuum psychometrically accessible. Subsequently, an extensive literature emerged that investigated associations between experienced meaning in life and mental health; an annotated bibliography of this work has been provided by Batthyány and Guttmann (2006). These studies are predominantly correlational in design and therefore permit no conclusions about the efficacy of therapeutic interventions, but they support the assumption that the experience of meaning is a clinically significant characteristic. More recent research on meaning has expanded these findings with independent instruments and models (Park, 2010; Steger et al., 2006).

A second line of research consists of controlled intervention studies. It should first be noted that studies on "classical" logotherapy in the narrower sense are comparatively rare and frequently show small samples, unclear manualization, or a lack of active control conditions. An important integrative contribution is the meta-analysis by Vos et al. (2015), which synthesized randomized controlled trials on various forms of existential therapy. The authors found indications of favorable effects, particularly for structured, explicitly meaning-centered interventions, whereas the evidence for other existential approaches was less clear-cut. At the same time, they emphasize the heterogeneity of the included studies and populations.

The methodologically best-investigated programs come from psycho-oncology. Meaning-Centered Group Psychotherapy, developed by Breitbart and colleagues and explicitly based on Frankl's work, was first examined in a randomized pilot study (Breitbart et al., 2010) and later in a larger randomized trial with an active control condition (Breitbart et al., 2015). In both studies, compared with supportive group psychotherapy, advantages emerged with regard to spiritual well-being and experienced meaning, as well as indications of a reduction in hopelessness and desire for hastened death. These findings are significant because they show that meaning-centered interventions can meet the standards of controlled research. However, they refer to a specific population and cannot readily be transferred to other clinical contexts.

4 Methodological Challenges

Several factors explain why the evidence base of logotherapy has remained narrow despite its long tradition. First, the logotherapeutic movement was for long stretches more firmly anchored in practice, teaching, and counseling than in university research. Second, the dual structure of anthropology and technique described above makes manualization more difficult. Third, the primary outcomes of logotherapy – experience of meaning, realization of values, attitude toward unalterable suffering – fit only to a limited extent with the symptom scales that usually serve as primary outcome criteria in disorder-specific efficacy studies.

In addition, there is a more fundamental question. Wampold and Imel (2015) contrasted the medical model of psychotherapy, which regards specific techniques as the main source of effects, with a contextual model that attaches considerable weight to the therapeutic relationship, plausible explanatory models, and the expectation of improvement. Grawe (1998), too, proposed a cross-school perspective with his common factors of change. From this point of view, the question of whether "logotherapy" as a brand-name procedure is effective would be less fruitful than the question of which of its elements contribute to change, and in what way. A review of the role of common factors, however, simultaneously

urges caution, since the evidence on these factors is also methodologically complex (Cuijpers et al., 2019).

A further problem concerns the comparison conditions. When meaning-centered interventions are compared with waiting lists or treatment as usual, nonspecific effects such as attention, structuring, and expectation cannot be separated from specific effects. Active control conditions, such as those used in the psycho-oncological studies (Breitbart et al., 2015), therefore represent methodological progress. Moreover, there is so far a near-total lack of studies that compare meaning-centered interventions directly with established disorder-specific procedures or that test their efficacy as a supplementary module within such procedures.

5 Perspectives for Process-Oriented Research

One way of turning the tension described into something productive lies in investigating mechanisms of change. Kazdin (2007) argued for systematically investigating mediators and mechanisms of therapeutic change instead of confining research to the comparison of treatment packages. For meaning-centered approaches, it seems natural to conceptualize changes in experienced meaning, in value orientation, or in the attitude toward unalterable life circumstances as possible mechanisms and to examine their temporal sequence in relation to symptom change.

This perspective fits into the debate on process-based psychotherapy, which places evidence-based processes rather than brand-name procedures at its center (Hofmann & Hayes, 2019). For logotherapy, such a framework offers the opportunity to translate its concepts into a common language with other procedures without abandoning its image of the human being. A prerequisite, however, is a precise operationalization of the postulated processes, a careful choice of instruments, and the willingness to take seriously results that do not conform to the hypotheses.

Concrete research questions arise from this perspective: Does an increase in experienced meaning precede the decrease in depressive symptoms in time, or does it follow it? Which patient groups benefit particularly from an explicit discussion of meaning and values? Under what conditions can the search for meaning, which in some studies is associated with distress (Steger et al., 2006), become therapeutically productive? Intensive longitudinal designs, systematic single-case analyses, and mediation models with repeated measurements are better suited to answering such questions than one-time pre–post comparisons.

6 Conclusion

The question of whether logotherapy is “evidence-based” cannot be answered with a simple yes or no. For the basic assumption that the experience of meaning is a clinically relevant characteristic, there is a broad, albeit predominantly correlational, body of findings. For manualized meaning-centered interventions in psycho-oncology, there are controlled studies with encouraging results. For logotherapy as a general psychotherapeutic procedure, by contrast, the evidence is limited. A sustainable research agenda should therefore aim less at legitimizing a school than at clarifying the mechanisms, indications, and limits of meaning-centered work.

For practice, this implies an attitude of well-founded modesty. Logotherapeutic concepts can enrich treatments, especially where questions of meaning, values, and coping with the unalterable are in the foreground. However, they should not be presented as a substitute for procedures whose efficacy for particular disorders is well documented. Such self-restraint, incidentally, corresponds to Frankl’s own understanding of logotherapy as a supplement to, not a replacement for, other psychotherapies (Frankl, 1946).

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